

Discharge Planning Tool

CMT: CMT-0013 (SR)

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General Discharge Planning Tool

- Transition of care planning may include(A)(2)(3):
 - **Assessment and early establishment of anticipated discharge destination and plans for care**, including(4)(5):
 - Treatment plan development involving multiple providers(6)
 - Premorbid functioning(7)
 - Patient and caregiver preferences and abilities(8)
 - Evaluation for skilled services at next level of care as appropriate for patient's continued needs
 - Psychosocial status. See Psychosocial Assessment [SR](#).
 - Social determinants of health status. See Social Determinants of Health Screening Tool [SR](#).
 - Acceptance for care for patient at next level of care, as appropriate
 - Discharge destination transition of care planning complete, as indicated(9):
 - Home considerations(10)
 - Home safety assessment need. See Home Safety Assessment [SR](#).
 - Patient safe to go home
 - Access to follow-up care
 - Self-management ability. See Activities of Daily Living (ADL) and Instrumental Activities of Daily Living (IADL) Assessment [SR](#).
 - Caregiver need, ability, and availability
 - Post-acute skilled care(11)(12)
 - Long-term acute care (LTACH)
 - Inpatient rehabilitation facility (IRF)/acute rehabilitation
 - Recovery facility care (eg, subacute or skilled nursing facility)(13)
 - Home care skilled services(10)
 - Palliative care(14)
 - End-of-life care(14)
 - Custodial care options include(15):
 - Long-term care facility
 - Assisted living
 - Group or care home
 - Private duty nursing or aide services
 - Respite care
 - Adult day care
 - **Transition of care plan complete**, which may include(6)(9)(16)(17):
 - Discharge destination identified and plans confirmed
 - Home plan completed, including:
 - Caregiver available and able to support patient care, or patient self-management confirmed
 - Home care arranged, if indicated(10)(18)
 - Admission evaluation complete and medical record transmitted to agency
 - Home care agency acceptance confirmed
 - First visit scheduled
 - Transportation arranged
 - Skilled care facility transfer (eg, recovery facility) plan completed, including:
 - Facility selected
 - Admission evaluation complete and medical record transmitted to next facility

- Facility acceptance confirmed
 - Medication reconciliation complete(19)
 - Admission date established and facility admission paperwork completed by patient or caregiver
 - Transportation arranged
- Long-term care (eg, nursing home, assisted living) transfer plan completed, including:
 - Facility selected
 - Admission evaluation complete and medical record transmitted to next facility
 - Confirmation of facility acceptance
 - Medication reconciliation complete(19)
 - Admission date established and admission paperwork completed by patient or caregiver
 - Transportation arranged
- Medical support needs addressed with providers and plan established
- Discharge summary communicated to all care providers at next level of care, including(12)(20):
 - Summary of main diagnosis and past medical history
 - Pertinent physical findings
 - Discharge medications with reasons for any changes from previous medication regimen
 - Adverse reactions to treatment (eg, allergies)
 - Results of procedures/tests and if any results pending (with plan for results to be communicated when available)
 - Prescribing providers information (eg, primary provider, specialty physicians)
 - Discharging provider contact information
 - Advance directive information(21)
- Medication reconciliation complete (19)(22)
- Appointments planned or scheduled:
 - Primary care provider(23)
 - Allergist/immunologist
 - Audiologist
 - Behavioral health provider(24)
 - Cardiologist
 - Cardiothoracic surgeon
 - Cardiovascular surgeon
 - Chemotherapy or radiation therapy
 - Dentist
 - Dermatologist
 - Diabetes care and education specialists
 - Dialysis
 - Dietitian
 - Endocrinologist
 - Follow-up evaluation call
 - Foot and nail care by trained care provider
 - Gastroenterologist
 - General surgeon
 - Gynecologist
 - Head and neck surgeon
 - Hematologist
 - Hepatologist
 - Infectious disease specialist
 - Interventional cardiologist
 - Interventional radiologist
 - Neonatologist
 - Nephrologist
 - Neurologist
 - Neuropsychologist
 - Neurosurgeon
 - Obstetrician
 - Oncologist
 - Ophthalmologist
 - Oral and maxillofacial surgeon
 - Orthopedic surgeon
 - Orthosis provider
 - Otolaryngologist
 - Pain management specialist
 - Palliative or end-of-life care provider

- Physiatrist or rehabilitation physician
- Plastic surgeon
- Podiatrist
- Prosthesis provider
- Psychiatrist
- Pulmonologist
- Radiation oncologist
- Rehabilitation therapy services
- Respiratory therapy
- Rheumatologist
- Specialists for management of comorbidities as needed
- Thoracic surgeon
- Transplant center or team
- Urologist
- Vascular surgeon
- Wound care center
- Wound, ostomy, and continence nurse (WOCN)
- Other: _____
- Testing and procedure plans made as needed:
 - Angiography
 - Bone densitometry
 - Bone marrow examination
 - Colonoscopy
 - Cystoscopy
 - Echocardiogram
 - Electroencephalogram (EEG)
 - Electrophysiologic studies
 - Endoscopy
 - Esophageal dilation
 - Esophagram
 - Exercise stress test
 - Infusion therapy
 - Laboratory testing
 - Notification of disease-control agencies
 - Nuclear medicine
 - Paracentesis
 - Radiology
 - Spirometry testing
 - Tilt table testing
 - Ultrasound
 - Other: _____
- Referrals made for assistance and support, which may include:
 - Alcohol and other drug abuse or dependence treatment
 - Behavioral health services (eg, counseling)(24)
 - Community services
 - Educational program (eg, condition management, self-management)
 - End-stage disease planning
 - Financial, for follow-up care, medication, and transportation
 - Home architectural modifications
 - Lactation specialist
 - Legal authorities, if indicated
 - Protective services
 - Self-help or support groups
 - Sexual counseling
 - Social services (eg, social programs, advance directives)
 - Special education
 - Tobacco use treatment
 - Vestibular rehabilitation
 - Vocational rehabilitation
 - Other: _____
- Medical equipment and supplies coordinated (ie, delivered or delivery confirmed) as indicated(25)
 - Adaptive devices (eg, feeding tools, reachers)

- Ambulation devices (eg, cane, crutches, walker)
- Antiembolic or compression stockings
- Apnea monitoring equipment
- Bath and toilet aids
- Blood pressure cuff
- Breast pump
- Chair lift
- Child safety seat
- Defibrillation vest
- Dialysis equipment and supplies
- Drain care supplies
- Enteral or parenteral nutrition supplies
- Fetal monitoring equipment
- Glucose monitoring supplies (eg, lancets, wipes, glucometer, test strips)
- Heart monitor
- Immobilizers (eg, braces, splints)
- Incentive spirometer
- Incontinence supplies
- Intravenous infusion supplies
- Lift equipment
- Lymphedema pump or compression garment
- Nebulizer
- Nutritional supplements
- Orthopedic equipment
- Orthoses
- Ostomy supplies
- Oxygen and supplies
- Peak flow meter
- Phototherapy
- Pressure injury prevention equipment
- Prostheses
- Pulse oximeter
- Respiratory equipment and supplies (eg, CPAP, BPAP)
- Rigid cervical orthosis
- Syringes and needles for subcutaneous injections
- Tracheostomy supplies
- Urinary catheter care supplies
- Urine 24-hour collection supplies
- Urine strainer
- Weight scale
- Wheelchairs and accessories
- Wound care equipment and supplies
- Other: _____
- Transition plan communicated to all members of patient's care team(20)(26)
- Patient and caregiver education completed, or ongoing education and support planned. See Education: Patient, Family, and Caregiver  SR.(27)
 - Basic condition, care plan, and other support needs information
 - Goals and expectations once discharged reviewed
 - Recovery expectations
 - Plan of treatment and importance of adherence
 - Follow-up care and importance of adherence
 - Transition to adult care providers, as needed(28)(29)
 - Preventive measures
 - Written and verbal discharge instructions on[B](31)(32):
 - Follow-up appointments
 - Provider name and contact information
 - Date, time, and location of appointment
 - Medication management(19)(22)(33)
 - Administration and regimen
 - Side effects, interactions, and adverse reactions[C]
 - Physician reporting, including reconciliation between providers(19)
 - Nutritional recommendations and restrictions

- Activity level and restrictions
- Self-management information; examples include:
 - ADL and IADL management
 - Comfort measures
 - Disease monitoring and management (eg, weight, blood pressure)
 - IV therapy management
 - Medical equipment (eg, DME) management
 - Wound care management
- Complication signs and symptoms and when to seek medical intervention(35)
- Patient and caregiver education complete. See Teach Back Tool [SR](#) for further information.(27)(36)
- Continued patient surveillance planned.(17)(31) See Post-Acute Care Follow-Up Call Tool [SR](#).

Evidence Summary

Background

Evidence suggests that a planned communication with the patient within 48 hours of discharge from acute care decreases readmissions. Planned communication is generally performed by a hospital discharge planner, health plan case manager, home health nurse, or pharmacist.(16)(17)

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Footnotes

[A] Health education materials should be given in patient's, family's, and caregiver's native language using trained language interpreters whenever possible.(1)(2) [A in Context Link 1]

[B] Patients with inadequate health literacy are more frequently hospitalized, have decreased ability to manage their condition, and are less adherent to medication regimens.(30) [B in Context Link 1]

[C] Naloxone rescue training and education about overdoses should be offered to the patient and family/caregiver, as indicated.(34) [C in Context Link 1]

Definitions

Medication reconciliation complete

- Medication reconciliation completion includes(1)(2):
 - Compare patient's discharge list of medications (prescribed and over-the-counter) against provider's admission or transfer orders.
 - Assess each medication for correlation to disease state or medical condition.
 - Report medication discrepancies to prescribing provider, attending physician, and primary care provider, and ensure accurate medication order is identified.
 - Provide reconciled medication list to all treating providers.
 - Confirm that patient or caregiver can acquire medication.
 - Educate patient and caregiver.

- Provide complete medication list to patient and caregiver.
- Importance of presenting personal medication list to all providers at each care transition, including all provider appointments
- Reason, dosage, and timing of medication (eg, use "teach-back" techniques)(3)
- Encourage communication between patient, caregiver, and pharmacy for obtaining prescriptions, setting up home medication delivery, and reviewing for drug-drug interactions.
- See Medication Reconciliation Tool for further information.

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Patient safe to go home

- Patient safe to go home; examples include(1)(2)(3):
 - Medical status stable for patient's condition
 - Functional care can safely be provided with available resources.
 - Mental status stable for patient's condition
 - Medication availability confirmed and reconciliation complete
 - Patient/caregiver education completed with written discharge instructions provided
 - Community resources identified and referrals made, as needed
 - Home care arranged, if indicated
 - Necessary medical equipment delivery arranged or available in home, if indicated
 - Necessary medical supplies ordered, or patient/caregiver can obtain, if indicated

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